

CHARLOTTE HUNGERFORD HOSPITAL

Outpatient Primary Care | 540 Litchfield Street, Torrington, CT 06790

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OUTPATIENT VISIT SUMMARY

Patient	DOB	Visit Date	Provider	Visit Type
Turner, Megan E.	06/06/1952	February 4, 2026	Steven Alford, MD	Quarterly Follow-up

CHIEF COMPLAINT / REASON FOR VISIT

Quarterly primary care visit. Post-bronchitis/COPD exacerbation follow-up (January 2026). Medication review. Blood pressure check.

VITAL SIGNS

BP	Heart Rate	Resp. Rate	O2 Sat (room air)	Weight	Temperature
132/78 mmHg	68 bpm	16/min	94%	136 lbs	98.4 F

SUBJECTIVE / HPI

Patient reports full recovery from January 2026 bronchitis. Completed prednisone taper and antibiotic course without issues. Nighttime oxygen therapy ongoing at 1L via nasal cannula since December 2025; patient reports improved sleep quality. Knee and hand pain stable, managing with acetaminophen and topical diclofenac gel. No chest pain. Appetite remains reduced but stable.

MEDICATIONS REVIEWED

Medication	Dose	Adherence
Tiotropium (Spiriva)	18 mcg daily	Adherent
Amlodipine	5 mg daily	Adherent
Albuterol (rescue)	PRN	Used 3x in January during exacerbation; none since 1/25/2026
Acetaminophen	650 mg up to TID PRN	Using 1-2 times daily
Diclofenac gel 1%	2g each knee BID	Adherent
Calcium + D3	600 mg / 400 IU daily	Adherent
Vitamin B12	1000 mcg daily	Adherent
Fiber supplement	Daily	Adherent
Nocturnal O2	1 L/min via nasal cannula	Using nightly

ASSESSMENT AND PLAN

1. Hypertension: Controlled. BP 132/78, consistent with home readings per patient log. Continue amlodipine 5mg. No adjustment needed.
2. COPD Stage II: Recovering well from January exacerbation. Oxygen therapy initiated December 2025 appears beneficial. Defer pulmonology to Dr. Chowdhury's February 18 appointment. Discussed awareness of exacerbation triggers (respiratory infections, cold air).
3. Osteoarthritis: Stable. Patient declining knee replacement at this time. Continue current regimen. Readdress at next quarterly visit.
4. General health: Weight 136 lbs, down 4 lbs since last year. Reviewed nutrition. Patient eating regularly but appetite reduced. No malabsorption concern at this time. Continue to monitor.
5. Next quarterly visit: October 2026 (confirmed by patient and David Turner).

Dictated and signed: Steven Alford, MD | February 4, 2026

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